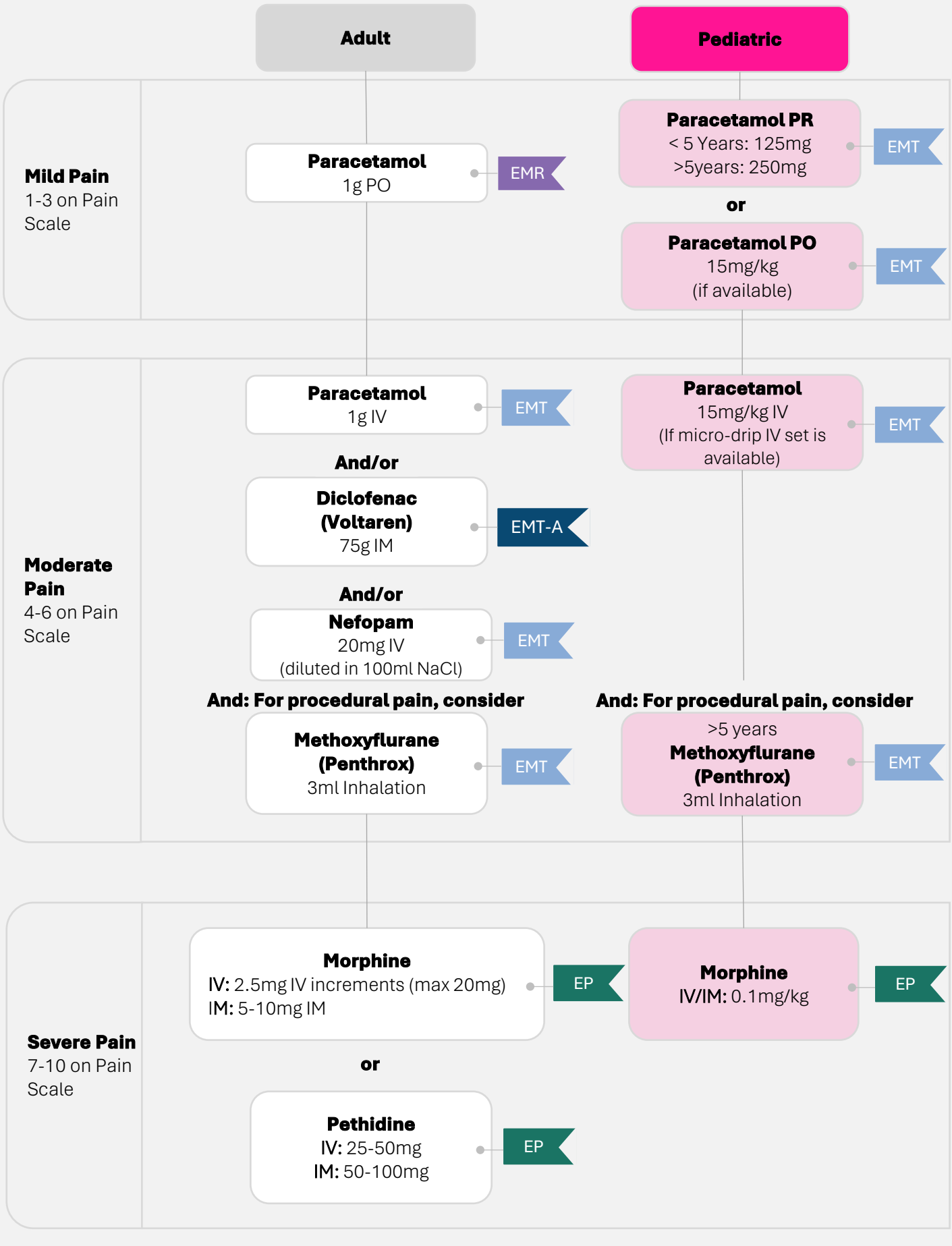
Adult
Pediatric
Mild Pain
1-3 on Pain Scale
Paracetamol
1g PO
EMR
Paracetamol PR
< 5 Years: 125mg
>5years: 250mg
EMT
or
Paracetamol PO
15mg/kg
(if available)
EMT
Moderate Pain
4-6 on Pain Scale
Paracetamol
1g IV
EMT
And/or
Diclofenac (Voltaren)
75g IM
EMT-A
And/or
Nefopam
20mg IV
(diluted in 100ml NaCl)
EMT
And: For procedural pain, consider
Methoxyflurane (Penthrox)
3ml Inhalation
EMT
Paracetamol
15mg/kg IV
(If micro-drip IV set is available)
EMT
And: For procedural pain, consider
>5 years
Methoxyflurane (Penthrox)
3ml Inhalation
EMT
Severe Pain
7-10 on Pain Scale
Morphine
IV: 2.5mg IV increments (max 20mg)
IM: 5-10mg IM
EP
or
Pethidine
IV: 25-50mg
IM: 50-100mg
EP
Morphine
IV/IM: 0.1mg/kg
EP

Patient Care Goals

1. Effectively manage acute pain to improve patient comfort and reduce distress.
2. Administer appropriate analgesics based on pain severity and underlying cause while minimizing adverse effects.

Patient Inclusion & Exclusion Criteria

Inclusion Criteria

- Patients presenting with acute pain of any origin (e.g., medical, traumatic, or unspecified), including:
  - Pain reported on a 0-10 scale (mild: 1-3, moderate: 4-6, severe: 7-10).
  - Pain associated with conditions such as chest pain, abdominal pain, burns, fractures, or sickle cell crisis.
  - Patients requiring pain relief to facilitate transport.
- Verbal or non-verbal indicators of pain (e.g., grimacing, guarding, or crying).

Exclusion Criteria

- Patients with no reported or observed pain.
- Pain clearly attributable to a chronic condition requiring specific non-prehospital management (e.g., chronic neuropathic pain without acute exacerbation).

Key Patient Assessment Points

1. **Pain Assessment**
   - Evaluate Pain Characteristics using OPQRST:
     - Onset, Provocation, Quality, Radiation, Severity, Timing.
   - Evaluate Pain Severity:
     - In **Adult Patients** and **Pediatric Patients > 3 years**:
       - Patients who can verbalize: Use a 0-10 pain scale.
       - Patient who cannot verbalize: Use the Wong-Baker FACES scale.
     - In **Pediatric patients <3 years**:
       - Use the FLACC pain scale.
   - Reassess pain score after interventions to evaluate response.
2. **Pain Source Identification**
   - Identify the underlying cause or trigger of pain (e.g., cardiac, musculoskeletal, visceral, or sickle cell crisis).
   - Note associated symptoms (e.g., nausea, shortness of breath, or fever) to guide treatment.
3. **Contraindications to Analgesics**
   - Assess for allergies to analgesics (e.g., morphine, paracetamol).
   - Identify conditions that may contraindicate specific medications (e.g., hypotension for opioids, liver disease for paracetamol).
4. **Functional Impact of Pain**
   - Evaluate how pain affects patient mobility, cooperation, or clinical stability (e.g., inability to lie flat for transfer due to pain).
5. **Determine Triage Category**
   - Determine Triage Category as per Patient Triage Categories CPG.
     - Note that pain severity does not influence Triage Category. However, severe pain is an indicator for a significant underlying issue.

Patient Management & Interventions

1. **Consider ALS back-up**
   - Consider the requirement for ALS back-up as per Universal Care CPG.
   - ALS back-up should be requested for Severe pain (7-10/10).
2. **Consider Red Flags**
   - Pain associated with life-threatening conditions (e.g., chest pain suggestive of myocardial infarction, abdominal pain with shock).
   - Hypotension (SBP <90 mmHg in adults, or age-specific for pediatrics) or respiratory depression.
3. **Evaluate for Signs of Shock or Instability**

- Hypotension, tachycardia, or poor perfusion (e.g., cool, clammy skin, delayed capillary refill >2 seconds).
- If shock is present, prioritize treatment of the underlying cause and use caution with analgesics (e.g., opioids in hypotensive patients).

4. **Consider Non-Pharmacological Interventions**
   - Positioning: Position the patient for comfort (e.g., semi-Fowler's for chest pain, splinting for fractures).
   - Reassurance: Use calm communication to reduce anxiety and perceived pain.
   - Cooling/Heat: Apply cold packs for acute injuries or heat for muscle spasms, as appropriate.
5. **Consider Establish IV Access**
   - Indicated for moderate to severe pain requiring IV analgesics or for patients at risk of deterioration.
   - Consider intraosseous (IO) access if IV access cannot be obtained
6. **Pharmacological Interventions**
   - **Mild Pain** (0-3/10)
     - **Paracetamol PO**
       - Adults: 1 g PO
       - Pediatrics:
         - PO 15 mg/kg (max 1 g) (if available)
         - PR <5years: 125mg
         - PR >5years: 250mg
   - **Moderate Pain** (4-6/10)
     - **Paracetamol IV**
       - Adults: 1g
       - Pediatrics: 15mg/kg (if micro-drip set available)
         - IV paracetamol is not recommended for children less than 33kg.
         - IV paracetamol generally should not be given to pediatrics unless a micro-drip administration set is available.
     - **Diclofenac**
       - Adults: 50-75 mg IM.
       - Pediatrics: Not recommended.
     - **Nefopam**
       - Adults: 20 mg IV, diluted in 100ml NaCl administered over 10 minutes.
       - Pediatrics: Not recommended.
   - **Moderate Pain with Associated with Painful Procedures**
     - Medications as per moderate pain, and/
     - **Methoxyflurane** (Penthrox)
       - Adults: 3ml Inhaled dose
       - Pediatrics: 3ml Inhaled dose
   - **Severe Pain** (7-10/10)
     - Medications as per moderate pain, and/
     - **Methoxyflurane** (Penthrox)
       - Adults: 3ml Inhaled dose
       - Pediatrics: 3ml Inhaled dose
     - **Morphine**
       - Adults:
         - IV: 2.5 mg increments, titrated every 5-10 minutes (max 20 mg).
         - IM 5-10mg.
       - Pediatrics:
         - IV: 0.1mg/kg (max 5 mg), titrated every 5-10 minutes (max 20 mg).
         - IM: 0.1mg/kg (max 10 mg).
     - **Pethidine**
       - Adults:
         - IV: 25-50mg. Single dose only.

        - IM: 50-100mg. Single dose only.
    - Pediatrics: 1 mg/kg IV/IM.

7. **Patient Reassessment**
    - Frequently Reassess the patient as per Universal Care CPG.
    - Specific Patient Reassessment Points:
        - **Monitor Pain and Response**
            - Reassess pain score every 5-15 minutes after interventions to evaluate efficacy.
            - Monitor for adverse effects (e.g., respiratory depression or hypotension).

Patient Safety Considerations

- ALS back up should be considered for patients with a pain score greater than 7.
- All patients who receive narcotic analgesia must be transported to the hospital for further evaluation and monitoring, regardless of symptom improvement at the scene

Clinical Insights & Additional Guidance

- **Pain Management Strategy**
    - Aim for adequate relief of pain that is satisfactory to the patient or best achievable under the circumstances.
    - Adopt a multimodal approach to pain management, combining multiple medications in addition to non-pharmacological interventions to enhance relief.
    - This strategy reduces reliance on any single medication and minimize side effects associated with high doses.
    - If pain relief is inadequate, proceed to the next step on the pain management ladder.
- **Clinical Judgment**
    - Decisions to give analgesia must be based on clinical assessment and not directly on a linear scale
    - Practitioners may make a clinical judgment and commence pain relief on a higher level of the pain ladder.
    - Base decisions to administer analgesia on clinical assessment, patient's overall condition, vital signs, and not solely on a linear pain scale.
    - Consider:
        - Patient's age, weight, and developmental stage.
        - Known allergies or contraindications.
        - Potential side effects and interactions, especially considering pediatric physiology.
- **Procedural Pain**
    - This is pain associated with painful procedures such as splinting or during movement over rough terrain. During these short times, pain can be severe, but the underlying pain is moderate.
    - It is encouraged to administer Penthrox for moderate pain in this circumstance.
- **Methoxyflurane** (Penthrox)
    - Provided adequate instruction and coaching is given Pentrhox can be an excellent pain reliever.
    - Penthrox is generally short acting – additional analgesia (such as IV paracetamol) should be administered in addition with Penthrox.
- **Prophylactic Antiemetics**
    - Routine administration of metoclopramide prophylactically for nausea and vomiting associated with morphine is not recommended.
- **Wong Baker Faces Pain Scale** (Non-verbal Adult & Pediatric Patients > 3 years)[11-13]

- **FLACC Pain Scale** (Pediatric Patients < 3 years)
  - Each of the 5 categories is scored 0 – 2.
    - Face – Legs – Activity – Cry – Controllability
  - The Total Score will be 0 – 10.
    - Pain Score:
      - 0 – Relaxed & Comfortable
      - 1 – Moderate discomfort
      - 2 – Moderate pain
      - 3 – Severe pain or discomfort or both.

FLACC Pain Assessment Table[14-16]			
Category	**0**	**1**	**2**
Face	No particular expression or smile	Occasional grimace or frown, withdrawn, uninterested	Frequent to constant quivering chin, clenched jaw
Legs	Normal position or relaxed	Uneasy, restless, tense	Kicking, or legs drawn up
Activity	Lying quietly, normal position, moves easily	Squirming, shifting back and forth, tense	Arched, rigid, or jerking
Cry	No cry (awake or asleep)	Moans or whimpers; occasional complaint	Crying steadily, screams or sobs, frequent complaints
Controllability	Content, relaxed	Reassured by occasional touching, hugging, or talking to; distractible	Difficult to console or comfort

Documentation

- Identification: Full patient name, age, and gender.
- Mechanism / Medical Complaint: Description of pain onset, severity, duration, and associated symptoms.
- Injuries / Information: Underlying cause or condition (e.g., fracture, sickle cell crisis), known diagnoses, and medical history.
- Signs: Pain score (pre and post treatment), vitals, AVPU/GCS, physical findings such as guarding or swelling.
- Treatment:
  - Route and dose of analgesic(s) administered.
  - Response to analgesics and reassessment of pain score.
- Allergies: Known allergies to medications like morphine, paracetamol, or NSAIDs.
- Medications: Current medications that may interact with analgesics.
- Background: Relevant chronic illnesses or contraindications (e.g., renal/liver disease).
- Other: ECG if indicated.

Key Performance Indicators

1. Document the pain score for all patients before and after administering analgesia.
2. Administer appropriate analgesia to patients with a pain score >7 to reduce pain score by at least 3 points.

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